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BNP
INOTROPE
NSAID

2.3 Complications of above = Decompensated Heart Failure

1) Flare gun next to floppy heart balloon:
Acute Decompensated Heart Failure = new or worsening signs & symptoms of HF, typically requiring ED and/or inpatient management

2) Mermaid blowing puff of air:
Acute **dyspnea** = most common symptom of ADHF **(Chief Complaint)**

3) Floppy, wet pant legs:
Worsening peripheral congestion (e.g., **ankle swelling**, abdominal fullness) is a typical symptom of ADHF **(Chief Complaint)**

4) Torn home med bag:
Nonadherence to CHF therapy (i.e., diet, meds) = most common precipitant of ADHF **(HPI)**

5) Heart-shaped guitar with broken string:
Myocardial ischemia is a common precipitant of ADHF **(HPI)**

6) Red laces on corset:
Conditions that cause **pulmonary hypertension** (e.g,. PE) can precipitate ADHF **(HPI)**

7) Steam coming out of ears:
Uncontrolled **HTN** is a common precipitant of ADHF **(PMHx)**

8) Broken kidney-shaped pulleys:
Renal failure is a common precipitant of ADHF **(PMHx)**

9) Home med bag:
Ask about use of cardiodepressant (e.g., beta blockers, calcium channel blockers) or cardiotoxic (e.g., NSAIDs, doxorubicin, trastuzumab) **medications**, which can precipitate ADHF **(Medications)**

10) Muted beta bugle:
Ask about use of cardiodepressant (e.g., **beta blockers**, calcium channel blockers) or cardiotoxic (e.g., NSAIDs, doxorubicin, trastuzumab) medications, which can precipitate ADHF **(Medications)**

11) Broken Calci-Yum ice cream machine:
Ask about use of cardiodepressant (e.g., beta blockers, **calcium channel blockers**) or cardiotoxic (e.g., NSAIDs, doxorubicin, trastuzumab) medications, which can precipitate ADHF **(Medications)**

12) NSAID fire extinguisher:
Ask about use of cardiodepressant (e.g., beta blockers, calcium channel blockers) or cardiotoxic (e.g., **NSAIDs**, doxorubicin, trastuzumab) medications, which can precipitate ADHF **(Medications)**

13) Box of rubies:
Ask about use of cardiodepressant (e.g., beta blockers, calcium channel blockers) or cardiotoxic (e.g., NSAIDs, **doxorubicin**, trastuzumab) medications, which can precipitate ADHF **(Medications)**

14) Tapestry:
Ask about use of cardiodepressant (e.g., beta blockers, calcium channel blockers) or cardiotoxic (e.g., NSAIDs, doxorubicin, **trastuzumab**) medications, which can precipitate ADHF **(Medications)**

15) Pulse ox-shaped lighthouse:
Many patients with ADHF will have relative hypoxemia **(SpO2 < 90% on room air)** due to pulmonary edema **(Vitals)**

16) Steam flying out of bulging smoke stack:
Hypertension (SBP > 140 mmHg) is more often seen in patients with ADHF **(Vitals)**

17) Sagging pipe with very little steam:
Hypotension (SBP < 90 mmHg) is rare in ADHF, but is a marker of severe disease and potential cardiogenic shock **(Vitals)**

18) Cracked heart-shaped spotlight next to lightning decal:
Hypotension (SBP < 90 mmHg) is rare in ADHF, but is a marker of severe disease and potential **cardiogenic shock (Vitals)**

19) Blue jug-shaped buoys:
ADHF can present with signs of systemic fluid overload (e.g.,**elevated JVP**, pretibial/ankle edema) **(Physical Exam)**

20) Wet, floppy pant legs:
ADHF can present with signs of systemic fluid overload (e.g., elevated JVP, **pretibial/ankle edema**) **(Physical Exam)**

21) Crackly velcro:
ADHF can present with signs of pulmonary edema (e.g., **diffuse crackles**, wheezing) **(Physical Exam)**

22) Loud party blower:
ADHF can present with signs of pulmonary edema (e.g., diffuse crackles, **wheezing**) **(Physical Exam)**

23) **S**lushi**3** cup:
S3 gallop is specific for HF **(Cardiac Auscultation)**

24) BNP blimp:
A BNP can be useful if the diagnosis of ADHF is uncertain **(Labs)**

25) Lung sails with skull & X-bones flag:
CXR in ADHF can reveal signs suggestive of heart failure (e.g., bilateral pleural effusions, pulmonary edema, and/or Kerley B lines)**(CXR)**

26) ECG-shaped fishing net:
Order **ECG** in patients with ADHF to evaluate for treatable cardiac precipitants (e.g., ACS, arrhythmias) **(ECG)**

27) Ultrasound megaphone:
Echocardiogram is recommended for most patients with ADHF, especially those with severe presentations (i.e., cardiogenic shock) **(Echo)**

28) DDx clipboard:
For further workup of Acute Decompensated HF, see Acute Dyspnea DDx

29) Weather murals:
Classify your patient according to **perfusion status** (i.e., warm / cold) and **fluid congestion** (i.e., wet / dry) **(1)**

30) Sun & rain:
The vast majority of patients with ADHF are adequately perfused **(“warm”)** and fluid overloaded**(“wet”)**

31) Snowflakes & rain:
The 2nd most common presentation of ADHF is **“cold”**(hypotension/shock) and **“wet”** (fluid overloaded)

32) O2 tank:
In all patients with ADHF, provide **supplemental O2** to maintain SpO2 ≥ 90% **(2)**

33) Loop de loop roller coaster:
Use IV **loop diuretics** (e.g., furosemide) to relieve fluid congestion as much as the patient’s BP allows **(3)**

34) Dilated blue pants:
Consider IV **venodilators** (i.e., nitroglycerin, nitroprusside, nesiritide) to relieve dyspnea in patients with ADHF who are NOT hypotensive**(4)**

35) Buff guy flexing biceps:
Give IV **inotropes** (e.g., dobutamine, dopamine, milrinone) to treat hypotension in patients with ADHF and reduced EF **(5)**

36) Ivy:
IV medications are used in the initial management of ADHF due to their fast onset of action